2.13 Acid–Base Imbalances

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I cannot get enough air." **Objective** • ABG: pH 7.28, $PaCO_2$ 58 mmHg • Drowsy • SpO_2 88%	**ACTUAL** **Impaired gas exchange** related to alveolar hypoventilation as evidenced by respiratory acidosis, hypoxemia and drowsiness.	ABG values moving back toward normal within 12 hours.	• Follow ABG, SpO_2 and level of consciousness as ordered. • Sit him upright so he can ventilate better. • Give controlled oxygen to the prescribed target. • No sedatives unless they are ordered. • If pH or $PaCO_2$ gets worse, be ready for non-invasive ventilation.	pH and $PaCO_2$ improving, patient alert: **goal met**.
Subjective • Feels sick and weak **Objective** • Breathing deep and fast • ABG: pH 7.25, HCO_3^- 14 mmol/L	**ACTUAL** **Ineffective breathing pattern** related to compensation for metabolic acidosis as evidenced by deep, rapid respirations.	RR 12–20/min as the acidosis is corrected.	• Watch the pattern and rate of breathing. • Treat the cause as prescribed (insulin and fluids, for example). • Bicarbonate only if it is ordered. • Keep a close eye on potassium.	Breathing pattern normal: **goal met**.
Subjective • *No specific complaint* **Objective** • Abnormal pH • Hypoxemia	**RISK** **Risk for acute confusion** as evidenced by risk factors: acid–base imbalance and hypoxemia.	Stays oriented.	• Check orientation every 4 hours. • Keep safety measures in place. • Report any change in mental state.	Oriented: **goal met**.